Supplementary table 1: Quality assessment of Seckel syndrome studies with a vasculopathy

| Study | Does the patient(s) represent(s) the whole experience of the investigator in the study (was the series specifically related to Seckel syndrome)? | Was the exposure (e.g conservative, pial synangiosis, endovascular treatment, surgical clipping) adequately ascertained? | Was the outcome (clinically and radiologically) adequately ascertained? | Was follow-up long enough for outcomes (ischemic stroke, aneurysm recurrence, or re-rupture) to occur? | Is the case(s) described with sufficient details to allow other investigators to replicate the research or to allow practitioners make inferences related to their own practice? | Quality |
|---|---|---|---|---|---|---|
| D'Angelo et al, 1998 [7] | Yes | Yes | Yes | Yes | Yes | High |
| Sorof et al, 1999 [8] | Yes | No | No | Yes | No | Moderate |
| Di Bartolomeo et al, 2003 [9] | Yes | No | No | No | No | Low |
| Codd et al, 2009 [4] | Yes | Yes | Yes | Yes | Yes | High |
| Rahme et al, 2010 [10] | Yes | No | No | No | Yes | Moderate |
| Inaloo et al, 2016 [11] | Yes | Yes | Yes | Yes | Yes | High |
| Gunesli et al, 2018 [12] | Yes | Yes | Yes | Yes | Yes | High |

* Reports with at more than three positive quality assessment criteria were considered low risk of bias, two or three positive qualities as medium risk, and one or none positive qualities was considered as high risk or bias.